HME922 · CLINICAL PRACTICE PASSPORT

Procedural Skills

OSCE Checklists

#	Procedure	Memory aid
1	Injections — IM / SC / ID	6 Rights · angles
2	Venepuncture	Order of draw
3	12-Lead ECG	4-4-5 · I SAL
4	Basic Life Support	DRSABCD · 30-2-100
4b	First Aid (HB6)	Burns · anaphylaxis · snake bite
5	Airway & Oxygen	4 O's · 4-5-15-15
6	Wound Dressing	TIME
7	IV Cannulation	Gauge colours · VIP

HOW TO USE

Procedural station = **one 10-minute station** (Assessment C, 10%). Any Year 1–2 procedure can come up.
Examiners mark the DOPS domains: **Clinical Knowledge · Consent · Preparation · Infection Control · Technical Ability · Patient Interaction · Documentation**.
Every procedure starts with the **Universal opening** and ends with the **Universal closing** — each page set prints on its own.

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

1 Injections — IM / SC / ID

HME922 Procedural Station · 10 minutes · Injections guideline + IM/SC DOPS form

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Right route, right site, right needle for the drug **and** the patient's body habitus. Check the **6 Rights**. IM: aspirate only by clinical judgement. SC: **never aspirate**.

UNIVERSAL OPENING

- ☐ Hand hygiene — Moment 1 DOPS
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers: full name, DOB, UR number/address**; check band + medication chart
- ☐ Explain procedure + why an injection (not oral) in plain language
- ☐ Ask about **allergies** (drug, latex, tape, skin prep) DOPS
- ☐ Allow questions DOPS
- ☐ Gain **informed verbal consent** DOPS
- ☐ Privacy, lighting, exposure, bed height up DOPS

MEMORY AID — 6 RIGHTS

Patient · **D**rug · **D**ose · **R**oute · **T**ime · **D**ocumentation — "Please Don't Disturb Rob's Tea, Doc"

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

CLINICAL KNOWLEDGE — SAY IT

- ☐ **Why inject:** drug not absorbed in GIT or too variable; route set by drug (e.g. lipid solubility) + manufacturer
- ☐ **Body habitus:** thick fat + short needle = accidental SC instead of IM
- ☐ **Check medication chart:** right drug, dose, patient, route, time DOPS

Complication	Prevent / manage
Local irritation, allergy, anaphylaxis	Check allergies to every ingredient
Infection	Hand hygiene, skin prep, infection control
Pain	Positioning + technique
Haematoma, bruising (anticoagulants)	Careful site, positioning, technique
Scar tissue / lipohypertrophy	Rotate sites (e.g. insulin)
Vessel injury	Know anatomy; aspirate if indicated
Nerve injury (esp. **sciatic**)	Landmark technique
Vaccine reaction	Observe at least **15 minutes**

CHOOSE NEEDLE & SITE

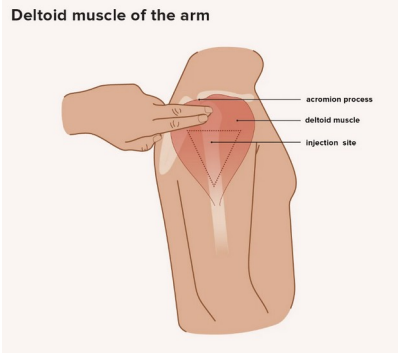
Route	Adult sites	Length	Gauge	Angle	Volume
IM	**Deltoid** · ventrogluteal (best if cachectic) · **vastus lateralis** · dorsogluteal (avoid if obese)	25–40 mm (76 mm large)	21–25 G	**90°**	1–5 mL syringe
SC	**Abdomen** (not within 50 mm of umbilicus) · thigh · upper outer triceps · upper buttock	Insulin 4–13 mm; other 13–16 mm	23/25 G	**45–90°**	≤ 1.5 mL (3 mL abdo)
ID (intradermal)	Anterolateral forearm · upper chest · upper back	10–19 mm	23/25 G	**5–15°** bevel up	0.1 mL (max 0.5)

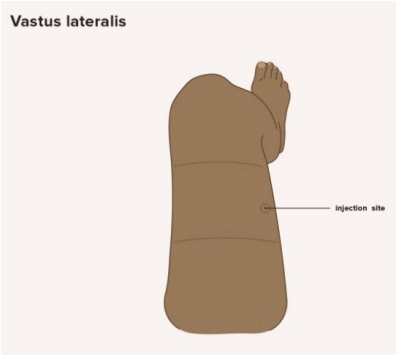
MEMORY AID — ANGLES

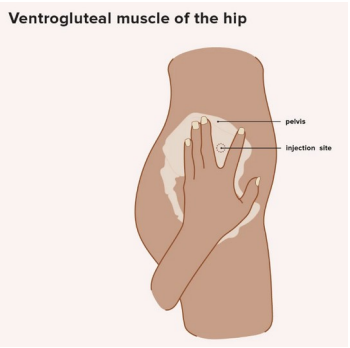
Route	Adult sites	Length	Gauge	Angle	Volume

Intra**M**uscular = **90** · **S**ub**C**ut = **45–90** · intra**D**ermal = **D**ead flat (5–15)
Getting deeper = getting steeper.

IM LANDMARKS



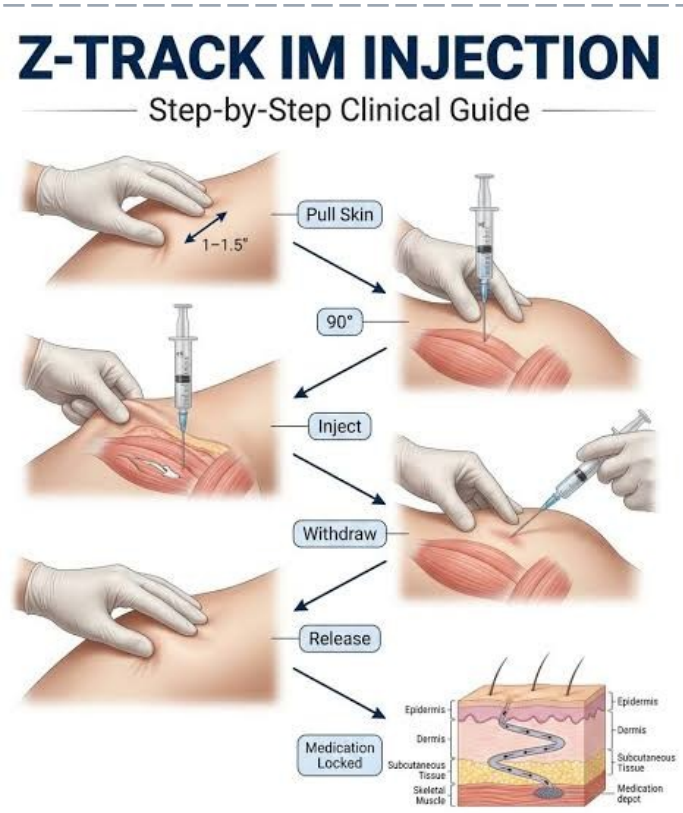




- ☐ **Deltoid:** sitting or hand on hip; expose whole upper arm; acromion + deltoid tuberosity; inverted triangle 2–3 fingers below acromion; inject centre
- ☐ **Vastus lateralis:** greater trochanter → lateral condyle; middle third, anterolateral thigh (rectus femoris = mid-anterior)
- ☐ **Ventrogluteal:** on side; heel of OPPOSITE hand on greater trochanter, index finger on ASIS, spread middle finger → "V"; inject middle
- ☐ **Dorsogluteal:** upper outer quadrant, 5–7.5 cm below iliac crest (crest, PSIS, ischial tuberosity, greater trochanter) — sciatic risk

PROCEDURE — IM (Z-TRACK)

1. Gather: 21–25 G needle, 1–5 mL syringe, swab, cotton ball/dressing, sharps bin
2. Hand hygiene + PPE; consent; 6 Rights
3. Locate site using landmarks DOPS
4. Swab site and **allow to dry** DOPS
5. Non-dominant hand pulls skin **2–3 cm sideways** (Z-track)
6. Dart needle in quickly and smoothly at **90°** DOPS
7. Aspiration **no longer routinely recommended** — clinical judgement by site; follow local policy
8. Inject smoothly at the drug's recommended rate
9. Withdraw needle **then** release skin (seals the track)
10. Cotton ball / dressing; needle to sharps DOPS



PROCEDURE — SC

1. 23/25 G needle, 1–3 mL syringe; select site and **rotate** sites
2. Swab, allow to dry
3. **No pinch** (short needle / enough fat) → 90°
4. **Pinch** (lift skin + fat off muscle) → 45–90°; 45° if cachectic
5. **DO NOT ASPIRATE**
6. Inject **slowly**; withdraw; cotton ball

PROCEDURE — ID

1. 1 mL syringe, bevel **up**, 5–15°
2. Insert only until bevel is covered
3. Inject slowly until a **bleb / wheal** forms
4. Uses: local anaesthetic, allergy testing, tuberculin (Mantoux), some vaccines

UNIVERSAL CLOSING

- ☐ Needle straight into sharps — **never recap**
- ☐ Waste: sharps / biohazard / general DOPS
- ☐ Area clean; patient comfortable; bed lowest; call bell DOPS
- ☐ Aftercare: report pain, swelling, rash, breathing difficulty (vaccines: wait 15 min)
- ☐ Hand hygiene — Moments 4/5 DOPS
- ☐ Document date, time, drug, dose, route, site, needle size, batch (vaccines), name + signature DOPS
- ☐ Can't answer? "I'll pass that on to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES

2 Venepuncture

HME922 Procedural Station · 10 minutes · Venepuncture guideline + DOPS form

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Feel for a soft, bouncy vein. Tourniquet off within **2 minutes**. Follow the **order of draw**. Max **2 attempts**. Label tubes **at the bedside**.

UNIVERSAL OPENING

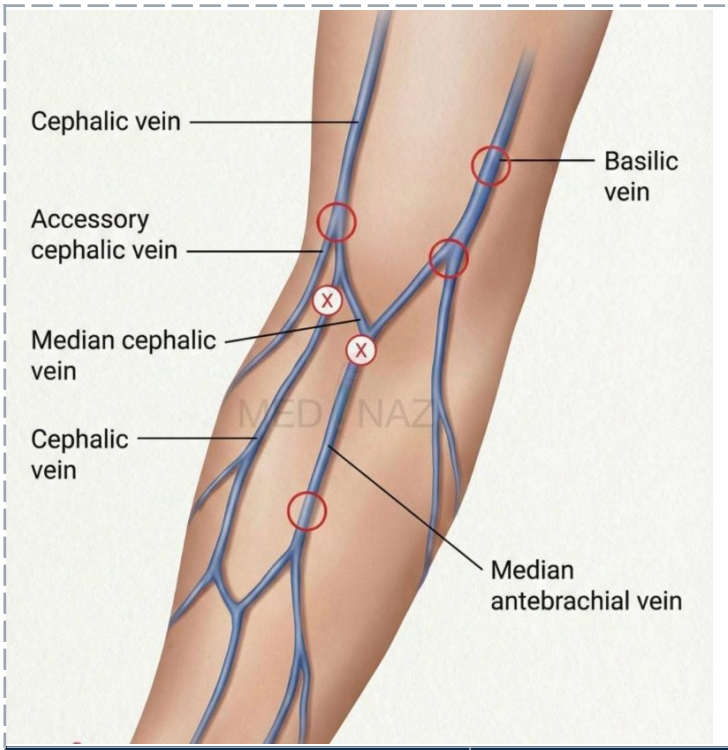
- ☐ Hand hygiene — Moment 1 DOPS
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers**; patient states name + DOB; check band against request slip DOPS
- ☐ Explain procedure + which tests
- ☐ Ask about **allergies** (latex, tape, alcohol) and **fainting history** DOPS
- ☐ Allow questions DOPS
- ☐ Gain **informed verbal consent** DOPS
- ☐ Privacy, lighting, exposure; seated/reclined, arm at or below heart DOPS

MEMORY AID — 5 MOMENTS

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

CLINICAL KNOWLEDGE — SAY IT

- ☐ Venepuncture = sample for lab tests. Venesection = therapeutic removal (haemochromatosis, polycythaemia)
- ☐ **Sites:** **cephalic, median cubital, basilic** — superficial, small nerve supply. Dorsum of hand = less prominent, more painful
- ☐ **Avoid:** lower limbs, small/non-palpable, repeatedly used, bruised/inflamed/sclerosed, infection, broken skin, cellulitis, phlebitis, lymphangitis, **arm with IV infusion**, limb for vascular surgery, **mastectomy side**, **AV fistula**; caution coagulopathy



Complication	Prevent/manage

Syncope	Fainting history → lie supine
Haematoma	Loosen tourniquet, remove needle, **firm pressure**
Infection	Uncommon — aseptic technique
Needlestick	Sharps bin at bedside, immediate disposal
Haemolysis	Falsely high **K+**; usually technique
Not enough blood	Paediatric tubes if difficult
Prolonged tourniquet	Falsely ↑ protein, AST, lipids, cholesterol, iron; affects PCV

PREPARE

- ☐ Check request slip for tubes; enough kit for **two attempts**
- ☐ Gloves (non-sterile, latex free), eye protection, sharps bin, kidney dish, **Vacutainer** (preferred closed system) / butterfly, alcohol swab, tourniquet, bluey, tubes, cotton ball + tape, labelled slip, specimen bag DOPS
- ☐ Difficult vein: **21 G butterfly**, or 21 G + 10–20 mL syringe
- ☐ Blood alcohol test → **only the kit swab**, no alcohol swab
- ☐ Same arm as IV fluid (no choice): stop infusion, wait **5–10 min**, sample distal to cannula

PROCEDURE

1. Tourniquet **10–15 cm above** site — fingers fit under it; arterial flow not blocked DOPS
2. Palpate (before gloves): soft, bouncy, refills, doesn't roll. **Pulsates = artery → don't use** DOPS
3. No vein? Fist clench/unclench; warm pack or warm water 5–10 min
4. Gloves; alcohol swab; **dry 30 seconds**; don't re-touch DOPS
5. Anchor vein — thumb below site, skin taut DOPS
6. Needle **bevel up, 10–30°**, in line with vein DOPS
7. Push tubes in, fill until vacuum stops; **gently invert** additive tubes DOPS
8. Tourniquet released by **2 minutes**
9. Tourniquet **fully off** → cotton ball → withdraw needle → firm pressure **1–2 min**
10. Bending the elbow is **NOT** pressure → haematoma
11. All tubes labelled **at the bedside**

> **MEMORY AID — ORDER OF DRAW — "Be Loving, Respectful, Gentle, Patient, Generous, Brave"**
> **B**lood cultures (sterile) → **L**ight blue (citrate) + ACD → **R**ed / gold (clot activator) → **G**reen (heparin) → **P**urple (EDTA) → **G**rey (glucose) → **B**lack (ESR) → other

TROUBLESHOOTING

Problem	Fix
No flow	Check position, advance slightly
Through the vein	Pull back slightly
Bevel against wall	Adjust angle
Vein collapsed	Tighten tourniquet, gentle fist
Haematoma forming	Release tourniquet, withdraw, firm pressure
2 failed attempts	Ask a senior colleague

UNIVERSAL CLOSING

- ☐ Needle straight into sharps — **never recap**

- ☐ Waste: sharps / biohazard / general DOPS
- ☐ Area clean; patient comfortable; bed lowest; call bell DOPS
- ☐ Check patient isn't light-headed before you leave; report bleeding/swelling
- ☐ Hand hygiene — Moments 4/5 DOPS
- ☐ Request slip dated, timed, signed; specimen bag with slip in outer pocket → lab DOPS
- ☐ Can't answer? "I'll pass that on to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES

3 12-Lead ECG

HME922 Procedural Station · 10 minutes · CR4 ECG guideline

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

10 electrodes → 12 leads. Find the 4th ICS, place V1–V6, record with patient still, **label the trace immediately**, then read it in 's fixed order.

UNIVERSAL OPENING

- ☐ Hand hygiene — Moment 1
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers**
- ☐ Explain: bare chest; some leads go **under the left breast**
- ☐ Ask about **skin/adhesive allergies**; brief history — why the ECG?
- ☐ Allow time for questions
- ☐ Gain **informed verbal consent**; offer a chaperone
- ☐ Private, **warm** room; supine; not over-exposed

MEMORY AID — 5 MOMENTS

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

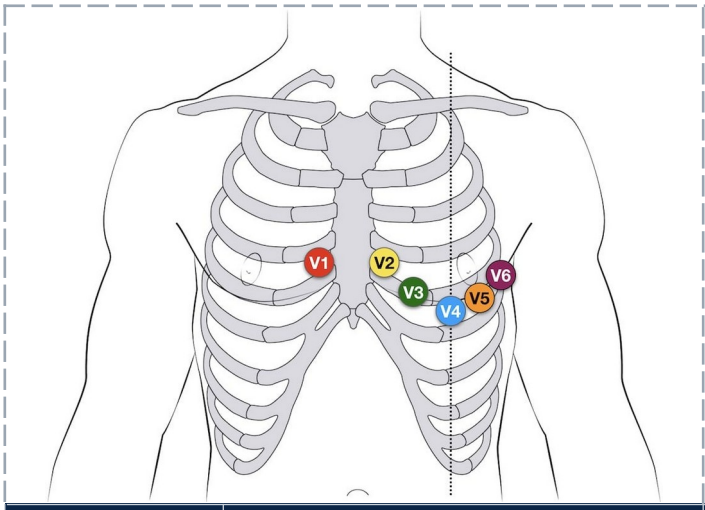
CLINICAL KNOWLEDGE — SAY IT

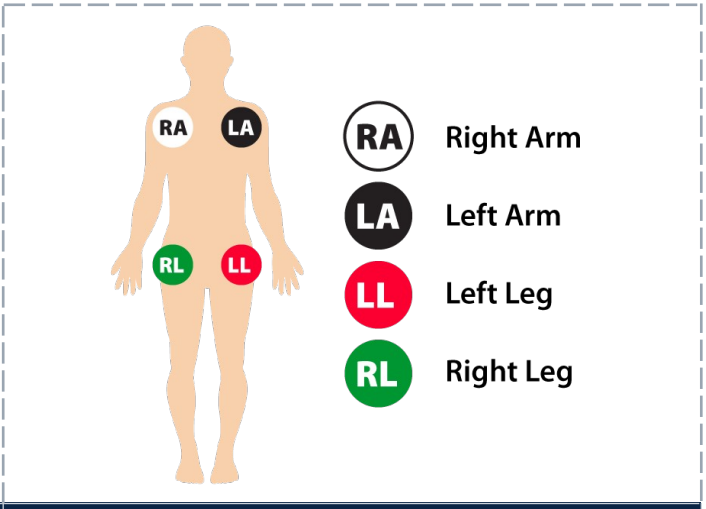
- ☐ Indications: chest pain · palpitations · heart failure · hypertension · SOB · congenital heart disease · electrolytes · drugs/toxins · routine medicals
- ☐ Detects: arrhythmia, ischaemia/AMI, conduction block (BBB), K+ disturbance, PE
- ☐ No alcohol, tobacco, caffeine or vigorous exercise in previous **30 minutes**
- ☐ RL = earth. I, II, III = bipolar (Einthoven). aVR, aVL, aVF = augmented. V1–V6 = unipolar, horizontal plane. 3/5-lead = rhythm monitoring only

PREPARE

- ☐ ECG machine charged + paper; paper towel; disposable clippers + sharps bin; sandpaper; electrodes; 10 labelled leads; tape
- ☐ Wipe sweat; **clip** hair (don't shave); sandpaper if needed

ELECTRODE PLACEMENT





Electrode	Position	Remember
V1	4th ICS, **RIGHT** sternal border	V1 & V2 = 4th ICS, either side

V2	4th ICS, **LEFT** sternal border	of the sternum
V4	5th ICS, mid-clavicular line	**Place V4 before V3**
V3	Midway V2 ↔ V4	the bridge
V5	Anterior axillary line, level of V4	5, 6 slide out sideways
V6	Mid-axillary line, level of V4	in a straight line
RA / LA	Inside right / left wrist	
RL / LL	Inside right / left ankle	RL = earth

MEMORY AID — PLACING CHEST LEADS

Find the **angle of Louis** → 2nd rib beside it → slide down to the **4th ICS** → V1 right, V2 left.

"**4-4-5**": V1 & V2 in the **4**th, V4 in the **5**th; V3 between; V5 & V6 follow V4 out to the side.

Limb colours (AHA, as in diagram): "**White on right, smoke (black) over fire (red), clouds over grass**". IEC machines: RA red, LA yellow, LL green, RL black — "**R**ide **Y**our **G**reen **B**ike" clockwise. Always check the lead labels.

Posterior (V7–V9) and right-sided leads are **not examinable**.

RECORD

1. Patient relaxed, still, not talking → **RECORD**
2. Rhythm strip prints (usually lead II)
3. Switch off, remove electrodes, patient dresses; wipe leads; machine back on charge
4. **Label the trace now:** name · DOB · date + **time** · chest pain / pain-free · vital signs · your name + signature

INTERPRET — ORDER

MEMORY AID — "Remember: Rate Rhythm, P, PR, QRS, ST, T, QT — then Compare"

Say the **patient details + presentation** first, finish by **comparing with an old ECG**.

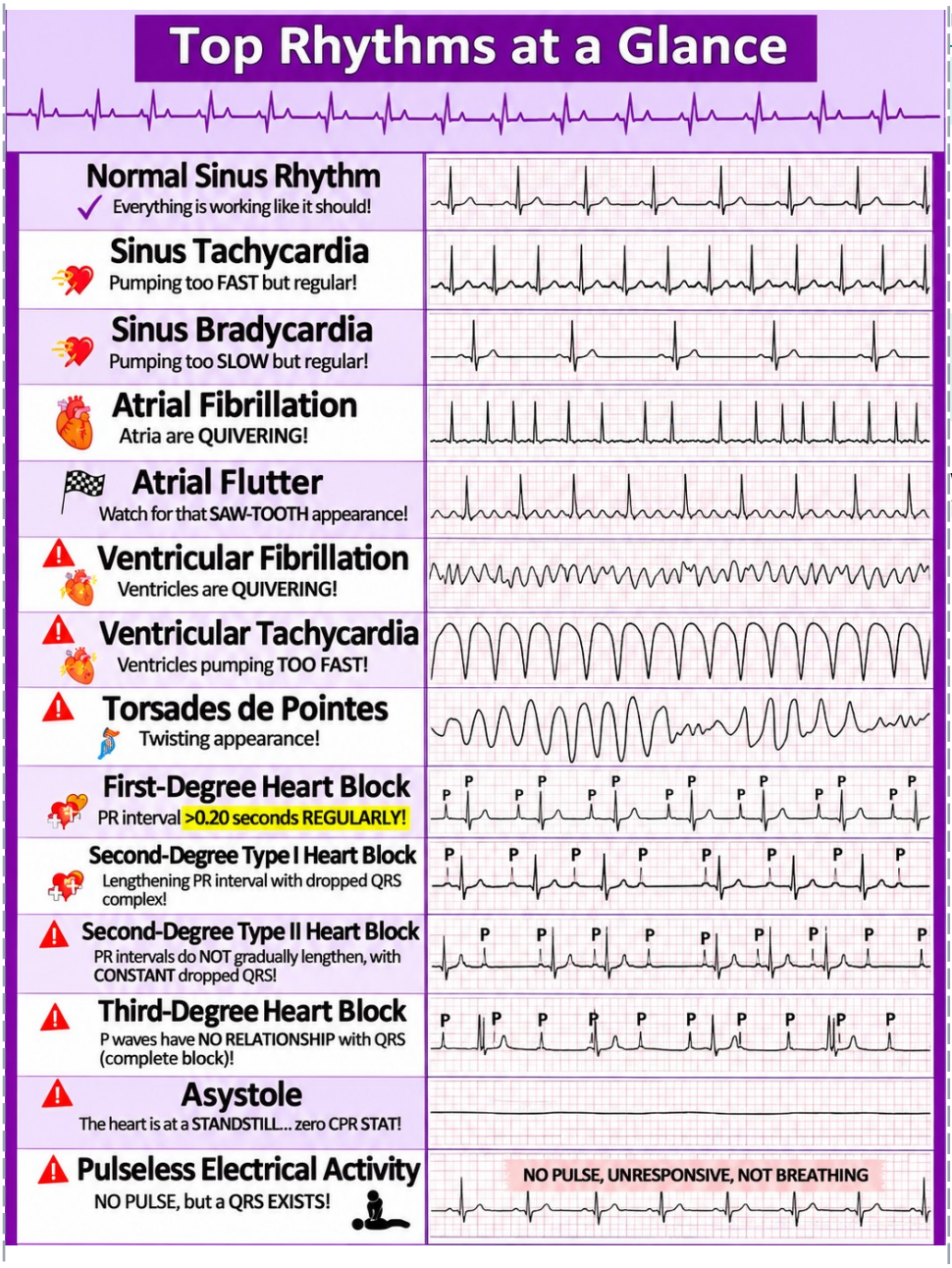
Step	Normal	Look for
Rate	60–100	300 ÷ large squares R–R; or QRS in 10 s × 6 (P waves × 6 = atrial rate)
Rhythm	Regular P–P and R–R	Every P followed by a QRS?
P wave	Upright, < 2.5 mm, < 0.12 s, 1:1	Absent (AF), saw-tooth (flutter)
PR	0.12–0.20 s (3–5 small sq)	Long = heart block
QRS	0.08–0.12 s	Wide (BBB); pathological Q > 0.04 s, > 1 mm, > 25% of R; R-wave progression
ST	Isoelectric	↑ > 1 mm limb / > 2 mm chest in 2 contiguous leads; ↓ ≥ 0.5 mm
T wave	Same direction as QRS; < 5 mm limb, < 10 mm chest	Inversion (TWI V1 can be normal); tall peaked = ↑K+
QTc	< 0.425 s	QTc = QT ÷ √RR; long QT → VT risk

TERRITORIES

MEMORY AID — "I SAL" — Inferior · Septal · Anterior · Lateral

Ischaemia = **repolarisation** (ST ↑ / ↓, T inversion). Infarction = **depolarisation** (pathological Q, small/notched QRS). **Time is muscle** — ~30 min ischaemia → infarction.

Territory	Leads	Artery
Inferior	II, III, aVF	RCA
Septal	V1, V2	LAD
Anterior	V3, V4	LAD
Lateral	I, aVL, V5, V6	LCx



UNIVERSAL CLOSING

- ☐ Clippers into sharps bin; electrodes into general waste
- ☐ Patient dresses; comfortable; bed lowest; call bell
- ☐ Explain next step — doctor will review; report chest pain immediately
- ☐ Hand hygiene — Moments 4/5
- ☐ Trace labelled + signed (check again)
- ☐ Can't answer? "I'll pass that on to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

4 Basic Life Support (Adult)

HME922 Procedural Station · 10 minutes · HB6 BLS & First Aid lecture + ANZCOR 2024

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Unresponsive and **not breathing normally** → follow the **ANZCOR BLS flowchart: DRSABCD**. Say every letter out loud. **30:2 at 100–120/min, 1/3 depth.** AED on **ASAP**.

UNIVERSAL OPENING (EMERGENCY VERSION)

- ☐ Introduce yourself to the examiner: name + "medical student"
- ☐ State: "Emergency — **consent is implied**"
- ☐ Apply **PPE — protective eyewear + gloves** before touching the patient
- ☐ Go straight to DRSABCD; talk the examiner through each step
- ☐ Patient wakes → introduce yourself, explain, reassure

MEMORY AID — DRSABCD — "Doctors' Rounds Start At Breakfast, Coffee 'n' Danish"

Danger · **R**esponse · **S**end for help · **A**irway · **B**reathing · **C**PR · **D**efibrillation

learning objectives: purpose of BLS · technical **and communication** skills for DRSABCD · **routine post-resuscitation care**

D — DANGER

- ☐ First priority: scene safe for **1 rescuer** (PPE: eyewear + gloves) · **2 patient** · **3 others** helping

Community dangers	Healthcare dangers
Violence · fire, smoke, fumes · traffic	Blood + body fluids · needles/sharps
Confined spaces · unstable structures	Wet floors / spilled fluids
Electricity · sharps · dangerous weather	**Electrical equipment** — don't touch a patient still connected to current
Deep/fast water · chemicals · biological	**Trip hazards** — move furniture/people to make space

R — RESPONSE ("TALK AND TOUCH")

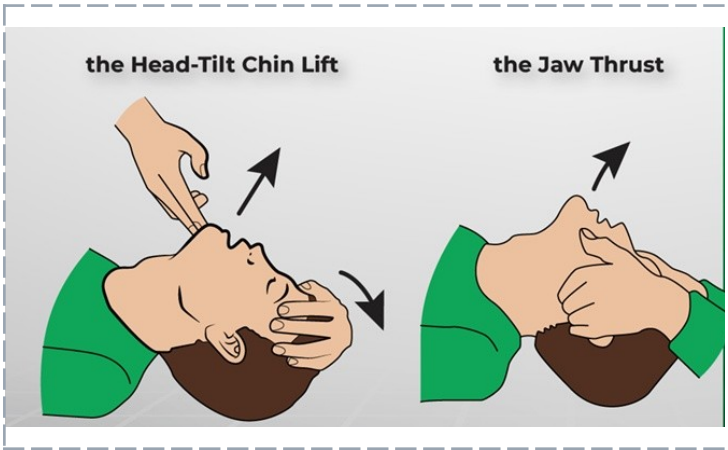
1. **Verbal:** "Can you open your eyes?"
2. **Touch:** "Can you squeeze my hand?"
3. **Painful:** firmly squeeze the shoulders (**trapezius**)

S — SEND FOR HELP

- ☐ **Delegate someone to get the AED**
- ☐ **In hospital:** call out — "**Hello, can I get some help in here please!**" · press emergency bell · dial **2222** (standard emergency number) → your **name, location (ward/bed)**, "**Code Blue**"
- ☐ **Out of hospital: 000**

A — AIRWAY (POSITION → OPEN & OBSERVE → OPTIMISE)

- ☐ Airway takes priority over **any** other injury, **including possible spinal injury** (ANZCOR 9.16)
- ☐ **Position:** adult + child = **sniffing** · infant = **neutral**
- ☐ **Open & observe:** can you see anything? Remove without pushing it further down — suction or head position. **No finger sweeps**
- ☐ **Optimise:** head tilt–chin lift or jaw thrust · oral airway adjunct (OPA) · suction



B — BREATHING

- ☐ Keep the airway open (manoeuvre / adjunct) **while** you check
- ☐ **Look** for chest movement · **Listen** for breath sounds · **Feel** breath on your hand
- ☐ Check for **no more than 10 seconds**; gasping/agonal breathing = **not normal**
- ☐ Unresponsive + not breathing normally → **start compressions immediately**

C — CPR

	Adult	Child	Infant
Hands	Two hands, lower half of sternum	One or two hands	Two fingers or encircling thumbs
Depth	**1/3** of chest	**1/3**	**1/3**
Rate	**100–120 / min** (~2 per second)	same	same
Ratio	**30 : 2**	30 : 2	30 : 2
Breaths	**1 second each**; mouth-to-mask or BVM; see chest rise		

- ☐ Rates **< 100 or > 140** are linked to lower survival (ANZCOR Guideline 6)
- ☐ Full recoil every compression; pauses < 10 s; swap compressor ~every 2 min

> **MEMORY AID — NUMBERS — "30 – 2 – 100–120 – 1/3 – 2 min"**
> 30 pushes, 2 breaths, 100–120 a minute, one-third deep, AED re-analyses every 2 minutes.
> Beat: "**Stayin' Alive**" ≈ 103 bpm.

D — DEFIBRILLATION (AED)

1. Only for someone **unresponsive and not breathing normally**
2. Attach **ASAP**, turn **on**, follow voice prompts
3. Remove clothing; attach pads (upper right chest + left side). **Pads touching?** → one middle of chest, one on back between shoulder blades
4. Plug pad cable into the AED
5. **Analysis: nobody touches the patient**

AED says	You do
Shockable	Charges (150 or 200 J) → **visual check** + loudly "**STAND CLEAR!**" → press shock → **CPR 2 minutes** with no interruptions
Non-shockable	Won't charge → "resume compressions" → CPR 2 minutes
Every 2 minutes	AED re-analyses — hands off

- ☐ Pads **not** over jewellery, pacemakers or medication patches
- ☐ Clip excess chest hair; wipe off moisture; pads firmly stuck
- ☐ Child **< 8 years** (incl. infants): paediatric attenuated pads (**50 J**) — or standard adult pads if none

> **MEMORY AID — SHOCK SEQUENCE — "Look, Shout, Shock, Push"**
> **Look** (visual check nobody touching) → **Shout** "Stand clear!" → **Shock** → **Push** (straight back to compressions)

CONTINUE BLS UNTIL

- ☐ **Signs of life** return
- ☐ **Qualified help** arrives and takes over
- ☐ **Impossible to continue** (e.g. exhaustion)
- ☐ An **authorised person pronounces death**

POST-RESUSCITATION CARE

- ☐ **Explain** what happened to the patient **and family**
- ☐ **Recovery position — left lateral**
- ☐ **Oxygen** if available
- ☐ Monitor closely: **reassess ABCDE** + vital signs (**HR, RR, BP**)
- ☐ To **ED** for observation + investigations (bloods, **ECG**, imaging) — **find the cause**, prevent recurrence
- ☐ Keep watching for further deterioration

CHOKING (AIRWAY GUIDELINE)

Severity	Action
Mild — effective cough	Sit up, encourage coughing, monitor, send for help
Severe — ineffective cough	Call for help; back blows then chest thrusts (up to 5 each), alternate; suction / forceps with caution
Becomes unresponsive	Start DRSABCD

UNIVERSAL CLOSING (EMERGENCY VERSION)

- ☐ **ISBAR handover** to Code Blue/ambulance: time of collapse, CPR start, number of shocks, return of circulation
- ☐ Sharps (if any) to sharps bin; waste sorted
- ☐ Recovery position (left lateral); oxygen; privacy; family informed
- ☐ Hand hygiene — Moments 4/5
- ☐ Document times, actions, shocks, outcome, name + signature
- ☐ Debrief with team; restock resus trolley / AED
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES

4b First Aid — Quick Reference

HB6 BLS & First Aid lecture · burns · trauma · anaphylaxis · snake bite

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Every first aid scenario starts with **DRSABCD** and **call for help (000)**. Then treat the specific problem, reassure, and stay with the person.

BURNS

- ☐ Clothes on fire → **STOP** (reassure) · **DROP** (to ground) · **ROLL** (wrap in blanket, roll until out)
- ☐ DRSABCD; severe burn or **airway burn** → 000
- ☐ **Cool running water 20 minutes**, as soon as possible
- ☐ Remove clothing + jewellery **unless stuck** to the burn
- ☐ Cover loosely with **cling film** or non-stick sterile dressing
- ☐ Check for and treat **shock**; calm and reassure
- ☐ **DON'T:** lotions, ointment, butter/fat or neutralising agents · touch the burn or burst blisters · remove stuck material · touch an **electrical burn** patient until power is safe

MEMORY AID — BURNS — "20 minutes, cool, cover"

Cool running water for **20 minutes**, then cover with cling film. Nothing greasy, nothing popped.

TRAUMA & SEVERE BLEEDING

- ☐ Signs of severe bleeding: **weak, rapid pulse** · pale, cool, moist skin · sweating · rapid gasping breathing · restless · nausea · thirst · dizzy/confused · loss of consciousness
- ☐ DRSABCD (move from danger if safe) · **gloves** · **000** · stay calm, stay with them
- ☐ Find injuries → **stop bleeding** (direct pressure) · **immobilise fractures** · keep **warm** · recovery position
- ☐ **DON'T remove embedded objects** · amputated parts **on ice** · **nothing to eat or drink** · minimise movement · elevate limbs (improves blood return to core in shock)

ANAPHYLAXIS

Mild–moderate	Severe (= anaphylaxis)
Swelling of lips, face, eyes	**Difficult / noisy breathing**
Hives / itchy raised rash	**Swelling of tongue / throat**
Anxious, scared	**Wheeze** → risk of LOC + cardiac arrest

1. DRSABCD
2. **Lay flat** — don't let them stand or walk (sit up only if breathing difficult)
3. Remove allergen if known (flick out sting, wipe food from mouth, stop medication)
4. Follow their **ASCIA Action Plan**; help them self-administer
5. **Adrenaline IM** — EpiPen® or Anapen® into the **outer (lateral) thigh**
6. **000** + family / emergency contact
7. Calm and reassure
8. **Further adrenaline** if no improvement
9. Stops breathing → DRSABCD

MEMORY AID — EPIPEN — "Blue to the sky, orange to the thigh"

Remove blue safety cap → orange end on outer thigh → push until click → hold **3 seconds**. Note the time. Lay flat.

SNAKE BITE

- ☐ Signs: pain, swelling, bruising, bleeding, puncture marks/scratches · tender groin/axillary glands · dizziness, nausea, vomiting, headache, abdo pain · **double/blurred vision, drooping eyelids**, difficulty speaking/swallowing · limb weakness/paralysis · breathing difficulty · collapse then apparent recovery
- ☐ DRSABCD · call for help (000)
- ☐ **Mark the bite site** with pen, cover with tissue (venom ID) — **don't wash it**
- ☐ **Pressure immobilisation bandage**, bandaging **upwards** the whole limb
- ☐ **Splint** the limb; keep person and limb **still**
- ☐ Reassure; stay until help arrives

> **MEMORY AID — SNAKE BITE — "Mark, Wrap, Splint, Still"**
> Mark the site → wrap the whole limb firmly upwards → splint → keep still (movement spreads venom via lymph).

MY NOTES

5 Airway Management & Oxygen Therapy

HME922 Procedural Station · 10 minutes · Airway Management & Oxygen Therapy guideline

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Work through the **4 O's**: **Observe** & clear → **Open** → **Optimise** with adjuncts → **Oxygenate** to a **documented SpO2 target**.

UNIVERSAL OPENING

- ☐ Hand hygiene — Moment 1
- ☐ Apply **PPE**
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers**
- ☐ Explain purpose of the assessment / oxygen
- ☐ Know the history (e.g. **COPD** → 88–92%); ask allergies (latex, tape)
- ☐ Allow questions; **informed consent** (unconscious = implied)
- ☐ Sit patient up if possible; warm, comfortable; privacy, lighting
- ☐ Stethoscope + airway equipment ready

MEMORY AID — THE 4 O's

Observe (FB, teeth, blood, vomit) → **O**pen (chin lift, head tilt, jaw thrust) → **O**ptimise (suction, OPA, NPA, LMA) → **O**xygenate (prongs, Hudson, NRB, BVM)

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

CLINICAL KNOWLEDGE — SAY IT

- ☐ Goal of O2: correct **alveolar and/or tissue hypoxia**
- ☐ Indications: respiratory distress · hypoxaemia · unwell/deteriorating incl. trauma · **SaO2 < 93%** · surgery · opiates/sedatives
- ☐ Targets: **92–96%** acutely ill · **88–92%** COPD / chronic respiratory failure
- ☐ Nurses may start O2 without an order, but follow the **target range documented by medical staff**; oximetry for all on O2; ABG/VBG if breathless + acutely ill
- ☐ Rising O2 need or no response = **deterioration** → urgent review, escalate

EQUIPMENT

- ☐ Stethoscope · torch · **Magill forceps** · suction + **Yankauer**/catheters · OPAs · NPAs · LMA/SGA · O2 wall/cylinder · **white flowmeter** · tubing · delivery devices

O1 — OBSERVE & CLEAR

- ☐ Look for foreign body, loose teeth, blood, vomit, saliva
- ☐ Mild obstruction: sit up, cough, monitor, send for help
- ☐ Severe: forceps with caution · lean forward + cough · back blows / chest thrusts · suction

O2 — OPEN

- ☐ Head tilt–chin lift
- ☐ **Suspected C-spine injury → jaw thrust ONLY**

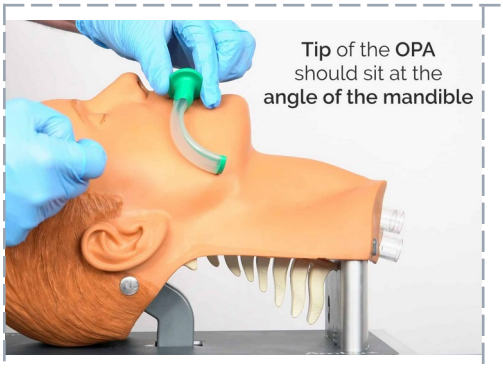
O3 — OPTIMISE WITH ADJUNCTS

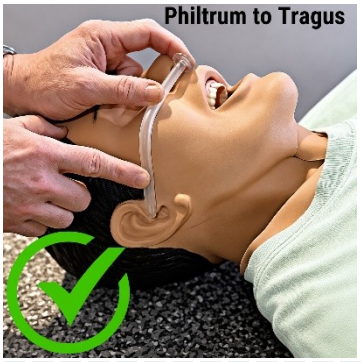
Adjunct	Who	Size	Key points
OPA (Guedel)	**Unconscious only**	Centre of top teeth → angle of mandible	Follow natural curve. Risks: aspiration, mouth/teeth trauma, wrong size → obstruction / laryngospasm
NPA	Awake or unconscious; better tolerated awake + children	Nares → tragus	Safety pin, lubricate, along nasal floor perpendicular to face, flange at nostril, tie with trache tape. **NO: basal skull # · mid-face # · coagulopathy**; caution septum/polyps
LMA / SGA	Unconscious; alternative to intubation	By weight; women 3–4, men 4–5	Lubricate, slide along back of throat until it stops, inflate cuff if needed, tape. **NO: aspiration risk**
Suction	Any	—	Wall outlet, canister, tubing, Yankauer

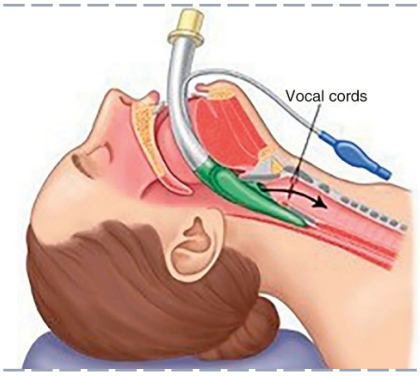
MEMORY AID — SIZING

OPA = Teeth → Jaw ("mouth tube, measure the mouth") · **NPA = Nose → Ear** ("nose tube, measure the nose")

OPA in an awake patient = gag/vomit → use an NPA.







BREATHING ASSESSMENT

- ☐ **LOOK** chest rise · **LISTEN** air at mouth/nose · **FEEL** air movement
- ☐ Observe **SpO2 · respiratory rate · colour**; perfusion, work of breathing

O4 — OXYGENATE

Device	Flow	FiO2	Key points
Nasal prongs	**1–4 L/min**	24 / 28 / 32 / 36%	≥ 5 L dries mucosa, crusting, epistaxis. Mouth breathing OK unless nares blocked
Hudson mask	**≥ 5 L/min**	6 L 45% → 10 L 65% (+5% per L)	Min 5 L stops CO2 rebreathing; FiO2 depends on flow, fit, effort; can humidify
Non-rebreather	**≥ 15 L/min**	60–90%	Emergency; spontaneous breathing. **Fill bag > 2/3 first.** No flow = **suffocation risk**
BVM	**15 L/min**	Up to 100%	Not breathing / inadequate breathing; self-inflating

MEMORY AID — FLOW RATES — "4 – 5 – 15 – 15"

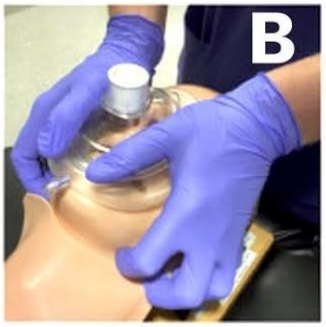
Prongs up to **4** · Hudson at least **5** · NRB **15** · BVM **15**

Prongs FiO2 = **20 + 4 × L** (1 L 24% … 4 L 36%) · Hudson: **6 L = 45%, add 5% per litre** to 10 L = 65%





Two-handed C-E grip



Two-handed vice grip

BVM TECHNIQUE

1. Mask: pointed end on bridge of nose, round end between lip and chin; tight seal
2. **C-E grip:** thumb + index = "C" on mask; 3 fingers = "E" lifting jaw into mask
3. Squeeze **1/3 of the bag**, **10–12 breaths/min**, watch chest rise
4. Over-inflation → gastric insufflation, volutrauma, barotrauma

PULSE OXIMETRY

- ☐ Normal **97–99%**; 93–97% may be normal for some; read as a **trend** with colour, perfusion, work of breathing
- ☐ False readings: cold peripheries / hypovolaemia · severe hypotension · cardiac failure · arrhythmia · poor probe · **CO poisoning** · nail polish

UNIVERSAL CLOSING

- ☐ Used adjuncts, catheters, masks into correct waste
- ☐ Area clean; patient sitting up, comfortable; bed lowest
- ☐ Simple explanation of findings; reassure
- ☐ Call bell — "**buzz** if you're concerned"
- ☐ Hand hygiene — Moments 4/5
- ☐ Document device, flow, SpO2, RR, target range, response
- ☐ Can't answer? "I'll forward that to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES

6 Wound Dressing (Aseptic Technique)

HME922 Procedural Station · 10 minutes · standard ANTT (check against wound guideline)

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Protect **key parts and key sites** with aseptic non-touch technique. Assess the wound systematically, clean **clean → dirty**, match dressing to wound, document.

UNIVERSAL OPENING

- ☐ Hand hygiene — Moment 1
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers**
- ☐ Explain why the dressing is being changed
- ☐ Ask about **allergies** (tape, iodine, chlorhexidine, latex) + pain/analgesia (give 30 min before)
- ☐ Allow questions
- ☐ Gain **informed verbal consent**
- ☐ Privacy, lighting, expose only the wound, bed height up

MEMORY AID — 5 MOMENTS

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

CLINICAL KNOWLEDGE — SAY IT

- ☐ Purpose: protect from infection/trauma, absorb exudate, moist healing environment, comfort
- ☐ Change when: care plan due, soiled/lifting/strike-through, signs of infection
- ☐ Infection signs: spreading redness, heat, swelling, pain, pus, odour, breakdown, fever

PREPARE

- ☐ Clean trolley (detergent/alcohol wipe, dry)
- ☐ Dressing pack, sterile 0.9% saline, sterile + non-sterile gloves, dressing, tape, waste bag, measuring guide, bluey
- ☐ Check **expiry** and packaging intact/dry

PROCEDURE

1. Hand hygiene → non-sterile gloves → remove old dressing; inspect it → discard with gloves
2. Hand hygiene → open pack by outer corners; drop saline + dressing onto field
3. Sterile gloves / forceps — **key parts touch nothing non-sterile**
4. **Assess:** site · size (L × W × D) · bed (granulation, slough, necrosis) · exudate · edges · surrounding skin · odour · pain
5. Clean **clean → dirty**; one swab, one stroke, discard
6. Pat surrounding skin dry
7. Apply dressing with 2–3 cm margin; secure without tension

MEMORY AID — WOUND ASSESSMENT — "TIME"

Tissue (bed) · **I**nfection / inflammation · **M**oisture (exudate) · **E**dges



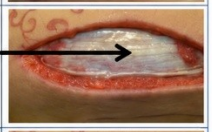
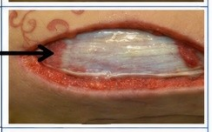
UNIVERSAL CLOSING

- ☐ Sharps to sharps bin — never recap
- ☐ Waste sorted; trolley cleaned
- ☐ Area clean; patient comfortable; bed lowest; call bell
- ☐ Report pain, bleeding, fever, redness, soaked dressing
- ☐ Hand hygiene — Moments 4/5
- ☐ Document assessment, dressing, next review; date on dressing
- ☐ Escalate infection/deterioration
- ☐ Can't answer? "I'll pass that on to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

Wound Bed Structures

SKIN & WOUND – QUICK REFERENCE GUIDE

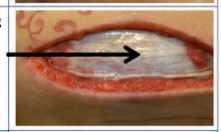
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Type of Tissue	Appearance	Example Photo
Fibrin	• Yellow tissue, firmly attached to wound bed, cannot be picked up with forceps. • Indicates normal healing. • Precursor to granular tissue and should not be debrided.	
Adipose	• Creamy, yellow. • Wet and soft when healthy. • Black/brown and can dry out when unhealthy.	
Fascia	• Connective tissue, shiny white, thin to thick depending on location. • Encapsulates structures such as nerves, bone, vessels, muscles and organs. • Should be shiny and wider than a tendon, as it wraps around the structure. Not as firm as tendon or bone.	
Muscle	• Striated, pink to dark red. • Firm and highly vascular.	
Tendon	• When healthy, appears as strong fibrous tissue, shiny white or yellow, attaches muscle to bone. • Unhealthy tendon may yellow and can appear like slough. • Should have attachment points on bone and muscle. If the client moves the nearest joint, the tendon should shift or move.	
Bone	• Hard, shiny, smooth, white appearance when healthy. • When unhealthy, can be brittle, rough and discoloured. • Often found in DFU and pressure injuries. • Will make a scraping sound when an instrument comes in contact with it. • If bone is palpable or visible, there is an increased risk of osteomyelitis, which can be very difficult to treat.	

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7 Intravenous Cannulation (PIVC)

HME922 Procedural Station · 10 minutes · MM3 guideline + IVC DOPS form

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Only if needed. **Smallest gauge** that works (largest in trauma). **2% chlorhexidine / 70% alcohol — dry 30 s — no touch.** Max 2 attempts. Date the dressing. VIP every shift.

UNIVERSAL OPENING

- ☐ Hand hygiene — Moment 1 DOPS
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers: name, DOB, UR/address**; patient states, check ID band DOPS
- ☐ Explain procedure, purpose, risks, benefits DOPS
- ☐ Ask about **allergies** (tape, chlorhexidine, alcohol) DOPS
- ☐ Ask: any difficult cannulation before? ("took the anaesthetist 4 attempts")
- ☐ Allow questions DOPS
- ☐ Gain **informed verbal consent** DOPS
- ☐ Privacy, lighting, exposure; **raise bed height** DOPS

MEMORY AID — 5 MOMENTS

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

CLINICAL KNOWLEDGE — SAY IT

- ☐ > **3,500** cannula-related sepsis cases/year in Australia (~1.5 / 1,000 admissions) — devices bypass body defences
- ☐ Indications: medications · fluids/electrolytes · blood products · unstable patient access · IV contrast
- ☐ Delivery: continuous (N/S, Hartmann's) · intermittent (antibiotics, electrolytes) · bolus (ondansetron, metoclopramide)

AVOID if possible	NEVER (unless life-threatening)
Cubital fossa in a stable patient	AV fistula
Legs / feet in adults (DVT, phlebitis)	Burns, scars, injured areas
Previous IV sites, bruising	Infiltrated or phlebitic veins
Dominant arm	Infected areas
Joints — wrist, elbow	Dermatological conditions
Mastectomy / axillary clearance side	Sclerosed or thrombosed veins

CHOOSE THE GAUGE

Gauge	Colour	mL/min	Use
14 G	Orange	240	Trauma, massive resuscitation, major surgery, peri-partum, GI bleed
16 G	Grey	180	As above
18 G	Green	90	Blood, irritant drugs, rapid infusion, CT contrast
20 G	Pink	60	General: fluids, antibiotics, analgesia
22 G	Blue	36	Paediatrics, fragile veins
24 G	Yellow	20	Paediatrics, neonates

MEMORY AID — GAUGE COLOURS — "Old Grey Goats Pick Blue Yams"

Orange 14 · **G**rey 16 · **G**reen 18 · **P**ink 20 · **B**lue 22 · **Y**ellow 24
Bigger number = smaller cannula = slower flow.



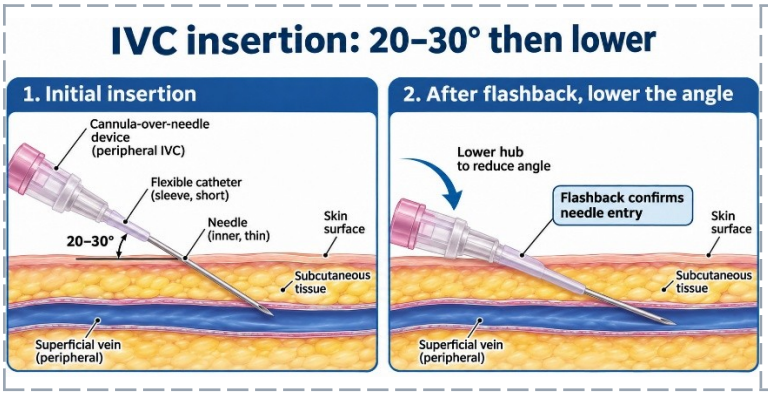
PREPARE

- ☐ Alcohol skin prep · IVC/dressing pack · tourniquet · transparent occlusive dressing · cannula · bung/valve · tape · bluey · gauze · sharps bin · splint/crepe if needed · 5–10 mL syringe · **0.9% saline flush** · blunt drawing-up needle · light · stool
- ☐ Standard precautions: hand hygiene, **gloves + safety glasses** DOPS
- ☐ Bluey under arm; you and patient both comfortable
- ☐ Vein: examine both arms; warm if needed; round, firm, elastic, well filled; straight length ≥ cannula

INSERT

MEMORY AID — KEY STEPS — "Clean, Dry, Bevel up, Flash, Lower, Slide, Flush"
Hand hygiene **before** opening sterile stock · ANTT throughout

1. Tourniquet on; locate vein DOPS
2. Skin prep **2% chlorhexidine in 70% alcohol** (aqueous if alcohol allergy); circular, inside → out ~5 cm; **dry 30 s**; clip hair, **never shave**; don't re-touch DOPS
3. Inspect cannula; rotate on stylet 360°
4. Anchor vein, skin taut, hand clear of insertion angle
5. **Bevel up**, with blood flow, **20–30°**, single smooth movement DOPS
6. **Flashback** → stop → lower angle almost to skin → advance a few mm DOPS
7. Slide **plastic cannula** off the needle to the hub — don't advance the needle; **never against resistance** DOPS
8. Release tourniquet; press over vein at cannula tip DOPS
9. Remove introducer → **straight into sharps**; attach bung while occluding DOPS
10. Bloods? Vacutainer onto bung, collect, then release tourniquet
11. **Flush 5–10 mL normal saline** DOPS
12. Transparent sterile dressing — **date/time sticker on it**; extra tape/splint DOPS
13. Failed? Retry only if confident + suitable vein — **max 2 attempts**



📷 Paste image here
Secured cannula with dated dressing

AFTER CARE — VIP SCORE

VIP	Findings	Action
0	Healthy	Observe
1	Slight pain OR slight redness	Observe
2	Two of: pain, erythema, swelling	**Resite**
3	All of: pain along vein, erythema, induration	Resite, consider treatment
4	Extensive + palpable venous cord	Resite, consider treatment
5	Extensive + **pyrexia**	Treat + resite (thrombophlebitis)

MEMORY AID — VIP
"**0–1 watch, 2 = resite**, 3–4 resite + think treatment, 5 = treat". Check **every shift**; remove when **no longer needed**.

☐ Complications: thrombophlebitis · thrombosis · extravasation (iron staining) · haematoma · nerve/tendon/ligament injury · infection — local (cellulitis) or systemic (CRBSI, endocarditis, osteomyelitis, sepsis, death)

UNIVERSAL CLOSING

☐ Introducer straight into sharps — **never recap** DOPS
☐ Waste: sharps / biohazard / general DOPS
☐ Area clean (no blood); patient comfortable; **bed back to lowest** DOPS
☐ Tell patient to report **pain, warmth, redness, swelling, bleeding** DOPS
☐ Hand hygiene — Moments 4/5 DOPS
☐ Document date, time, site, gauge, indication, clinician name DOPS
☐ Can't answer? "I'll pass that on to someone senior"
☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES